Physician, patient, team and law

The chronically ill patient, models of the physician–patient relationship, interprofessional relations and the physician's legal liability — with seven cases

Practical Class 4 · Social Medicine and Medical Ethics I · 2026/2027

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Plan of the class

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1 · The chronically ill patient	1
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The thread of the class

- **trust** is built in the relationship, in the team and in the institution
- the **law** defines the consequences when trust is broken
- one professional act can lead to **four kinds** of liability at once
- quotations of Bulgarian acts are working English translations

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1. **The chronically ill patient**
2. Models of the physician–patient relationship
3. Interprofessional relations
4. The physician's legal liability

Chronic illness changes the logic of care

Dimension	Acute care	Chronic care
Time	a limited episode	long-term or lifelong follow-up
Goal	control the danger, restore health	control, preserved function, acceptable quality of life
Patient	receives help, follows a short plan	makes daily decisions and carries out much of the care
Physician	diagnoses and acts promptly	sustains the relationship, agrees goals, coordinates
Typical model	guidance — cooperation	mutual participation, agreed plan
Main ethical risk	acting without consent under time pressure	control of the person in the name of control of the disease

Disease is the diagnosis; illness is the experience of it. Good chronic care addresses both.

Coping and reactions to chronic illness

Coping strategies

- **problem-focused:** information, self-monitoring, planning, asking for help
- **emotion-focused:** distancing, distraction, sharing feelings, faith
- **meaning-focused:** reordering goals, new roles, values beyond the illness

Biographical disruption (Bury 1982)

Chronic illness can disrupt identity, relationships and anticipated life plans — life splits into "before" and "after". Adaptation is not resignation.

Reaction	Looks like	Professional response
Denial	"It isn't serious for me."	information in stages; check for dangerous omissions
Anger	blaming the team or relatives	name the emotion; do not take it personally
Regression	passivity, total dependence	preserve skills; small agreed tasks
Hopelessness	"There is no point."	assess pain, depression and self-harm risk

Folkman S, Moskowitz JT. Annu Rev Psychol 2004;55:745–774. doi:10.1146/annurev.psych.55.090902.141456 · Bury M. Sociol Health Illn 1982;4:167–182. doi:10.1111/1467-9566.ep11339939

Autonomy, adherence and treatment burden

Relational autonomy

Dependence on a machine, drugs or relatives does not remove the right to choose. The choice may still be shaped by guilt, fear or pressure — the physician helps the patient decide **for himself**.

Adherence, not "compliance"

Adherence is agreement between behaviour and an **agreed** plan. Non-adherence may be **unintentional** (forgetting, complexity, cost) or **intentional** (side effects, fear, a different view of benefit). Look for the cause, not someone to blame.

Treatment burden

drugs, measurements, diets, visits, travel, costs, paperwork. With several chronic diseases, guideline-by-guideline care can become an unworkable plan. Ask: **how much more can this person carry?**

Shared responsibility

physician: a sound, simple, explained plan · **patient:** honest feedback about what he actually does · **system:** accessible care and continuity

The family: support, burden and limits

What the family brings

- daily care and observation of changes
- knowledge of the patient's life and values
- emotional and practical support, transport, medicines

What the risks are

- speaking **for** the patient; filtering information
- overprotection and loss of skills
- exhaustion of the main carer ("caregiver burden")
- neglect or abuse — see the patient alone

> *The patient has the right to authorise in writing another person to see his medical documents and to make copies of them.*
>
> — Health Act, Art. 28b(2) (Class 3)

The patient decides which relatives take part and what they learn.

Case 1 · My doctor is an old friend

The case

A man with newly diagnosed hypertension has known his general practitioner since childhood. Two months into treatment his blood pressure is higher than at diagnosis. He takes the tablets "when I feel the pressure" and says, laughing: "We've known each other for years — just prescribe me something stronger." He has not recorded the agreed home blood-pressure readings or had the agreed blood tests.

1. What explains his behaviour — lack of discipline or his model of the illness?
2. Which relationship model fits, and what does the friendship change?
3. What exactly should the doctor do at this visit?

Case 1 · My doctor is an old friend — key points

Ethics

- "Pressure that I feel" is a lay model of illness: hypertension is mostly silent. Intentional non-adherence follows from it.
- Familiarity hinders an honest history and clear agreements; "something stronger" is a request, not an indication.
- Model: **mutual participation** (Szasz–Hollender) — agreed goals, measurements, review dates. A **deliberative** approach (Emanuel) adds a talk about why preventing silent long-term harm matters to him. The friendship invites the untested assumption of shared goals that Veatch criticises in the collegial model.

Practice and limits

- Explore his beliefs without blame; teach-back; a simpler regimen; home measurements.
- Measure blood pressure now and look for acute target-organ damage. If he is clinically stable, check adherence and confirm measurements before intensifying treatment.
- If the friendship obstructs treatment, offer another physician — without abandoning him (Code, Art. 24(1): continuity on withdrawal).

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2. **Models of the physician–patient relationship**
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Models of the physician–patient relationship: an overview

What a model is

- an **ideal type**: how information, authority and responsibility for the decision are shared
- a tool for analysis, not a label for a physician's character
- one consultation can mix models; the appropriate model can **change** during one illness

What every model answers

1. who sets the goals of care?
2. whose values guide the decision?
3. who makes the decision — and who bears responsibility?

Classification	The question it asks
Szasz & Hollender (1956)	how far can the patient **take part**, given his clinical condition? — three models
Veatch (1972)	who holds **ethical authority** in the relationship? — four models
Emanuel & Emanuel (1992)	how does the physician deal with the patient's **values**, and what does **autonomy** mean? — four models

The criteria differ, so the models of different authors do not correspond one-to-one. Shared decision-making (later in this block) offers a practical approach to these questions.

The model describes how roles are shared — and it can change during one illness.

Three classic typologies

Author	Models	Key idea
Szasz & Hollender (1956)	activity–passivity · guidance–cooperation · mutual participation	the degree of the patient's participation follows his **condition** (coma → acute illness → chronic illness); the error is to keep the patient passive once he can take part
Veatch (1972)	engineering · priestly · collegial · contractual	the engineering model ignores values, the priestly model is paternalistic, the collegial model assumes shared goals that are often missing; Veatch **favours** the **contractual** model — shared ethical authority, both keep their moral integrity — though a contract does not exhaust the moral duty
Emanuel & Emanuel (1992)	paternalistic · informative · interpretive · deliberative	models differ in how they understand the **patient's values**; the **deliberative** physician discusses which health-related values are worth pursuing and persuades without coercion; the authors favour it as the general ideal

Szasz TS, Hollender MH. AMA Arch Intern Med 1956;97:585–592. doi:10.1001/archinte.1956.00250230079008 · Veatch RM. Hastings Cent Rep 1972;2(3):5–7. doi:10.2307/3560825 · Emanuel EJ, Emanuel LL. JAMA 1992;267:2221–2226. doi:10.1001/jama.1992.03480160079038

Paternalism and professional limits

Strong paternalism

overrides an informed patient with capacity — concealing a diagnosis, ignoring a refusal: a strong ethical presumption **against** it.

Weak paternalism

protects a person whose capacity for **this** decision is impaired, only while that ground exists. Treatment without consent still needs a **specific legal basis** (Health Act, Arts 87, 89(2), 90(4), 91).

> *Persons practising a medical profession have the right to freedom of action and decision according to their professional qualification, medical standards and medical ethics.*
>
> — Health Act, Art. 190(1)

> *Regardless of his specialty, qualification or commitments, the physician must provide emergency care to every patient whose health and life are in immediate danger.*
>
> — Code of Professional Ethics, Art. 11

> *The physician may refuse treatment and consultation when he is convinced that the necessary trust between him and the patient is lacking ... This does not apply to his duty to provide medical care in emergencies.*
>
> — Code of Professional Ethics, Art. 12(1)

The patient may refuse offered treatment; he cannot demand treatment without a reasonable clinical indication.

Shared decision-making

1 · Choice talk

"There is a choice to make, and I would like us to make it together." Support the patient's deliberation.

2 · Option talk

the reasonable options — including waiting or doing nothing — with benefits, harms and uncertainty, in plain numbers

3 · Decision talk

"What matters most to you?" A reasoned recommendation; a decision; check understanding; agree follow-up

> *As a patient, everyone has the right to ... more than one medical opinion on the diagnosis, treatment and prognosis ... clear and accessible information about his state of health and the methods of possible treatment.*
>
> — Health Act, Art. 86(1)(4) and (8)

> *The physician respects the patient's right to request a second opinion and information about other methods of diagnosis and treatment.*
>
> — Code of Professional Ethics, Art. 28(2)

Elwyn G, Frosch D, Thomson R, et al. Shared decision making: a model for clinical practice. J Gen Intern Med 2012;27:1361–1367. doi:10.1007/s11606-012-2077-6

Truth-telling and breaking bad news

> *The patient has the right to refuse to be informed about the disease, its prognosis and the planned activities, except where his state of health endangers the health of others. The decision is recorded in writing in the medical documentation. The patient may authorise in writing a person to be informed instead of him.*
>
> — Health Act, Art. 92(2)–(4) (abridged)

A family asks for concealment

Explore the fear; ask the **patient** how much he wants to know and who should be present. The family's wish does not replace his.

	SPIKES — a memory aid
S	**Setting:** privacy, time, the right people, sit down
P	**Perception:** what does the patient already know?
I	**Invitation:** how much does he want to know?
K	**Knowledge:** warning shot; plain words; small parts
E	**Emotions:** name and acknowledge them; allow silence
S	**Strategy and summary:** next steps, support, follow-up

Baile WF, Buckman R, Lenzi R, et al. SPIKES — a six-step protocol for delivering bad news. Oncologist 2000;5:302–311. doi:10.1634/theoncologist.5-4-302

Professional boundaries

Crossing (may be acceptable)	Violation (not acceptable)
a modest, customary gift shared with the team	an expensive, secret or conditional gift
attending a patient's funeral	a romantic or sexual relationship with a patient
a short reply on an official channel	persistent personal messaging that creates dependency or serves the physician's needs
emergency help to a relative	continuing to treat and prescribe for family informally

> *Respect for the intimate sphere of the patient is necessary. It is impermissible for a physician to use the examination and treatment to engage in intimate contact with the patient.*
>
> — Code of Professional Ethics, Art. 27

Test for any crossing

purpose · patient's vulnerability · value · secrecy · reciprocity · effect on judgement · how it would look in the record or to a colleague

Case 2 · The diagnosis the family wants hidden

The case

Mrs D., 67, comes with her son to receive a biopsy result. Before the consultation the son asks the oncologist to say only that "an inflammation" was found: the word **cancer** would destroy his mother's hope. Mrs D. asked for her son to attend, but never asked him to receive the information **instead** of her. During the meeting she looks at the doctor: "Is this cancer, and can it be treated?" The son interrupts: "More tests are needed."

1. What do we know about **her** wishes, and what must not be inferred from his request?
2. How can the oncologist acknowledge the son without colluding in deception?
3. Formulate the next three sentences.

Case 2 · The diagnosis the family wants hidden — key points

Law

- The treating physician must inform the **patient** (Health Act, Art. 92(1)); Code Art. 28(1).
- Only the patient may decline information — the refusal is recorded in writing in the medical record (Art. 92(2)–(3)) — or authorise **in writing** a person to be informed instead of her (Art. 92(4)). The son has no such authorisation.
- "Inflammation" would be deception: it would prevent properly informed consent to the proposed cancer treatment (Arts 87–88; Class 2).

Ethics and practice

- Speak to her: "You are asking me directly, so I will answer you directly. The biopsy shows a cancer. There is treatment, and we will plan it together."
- Acknowledge the son: "I can see how much you want to protect your mother."
- Check what she wants to know now; give information in parts; allow silence; agree the next step.
- Model: truthful, autonomy-supporting communication — **deliberative** (Emanuel) when they also discuss which health-related values should guide treatment. The son asks for **paternalistic** concealment; in Veatch's terms, for a **priestly** role.

Case 3 · The operation the patient demands

The case

Mr I. has chronic back pain without neurological deficit. Two specialists recommend rehabilitation and explain that surgery is unlikely to help and carries a substantial risk. He insists that the neurosurgeon operate: "Autonomy means the choice is mine." He threatens a complaint and mentions a private clinic abroad that advertises the operation.

1. What does respect for autonomy require — and what does it not require?
2. Is a firm refusal paternalism?
3. How can the surgeon keep him in safe care?

Case 3 · The operation the patient demands — key points

Law and limits

- Autonomy is the right to **accept or refuse** offered care (Health Act, Arts 87, 90), not a right to any intervention.
- The surgeon acts according to qualification, standards and ethics (Art. 190(1)); performing an operation without an indication could itself engage liability.
- He has a right to another medical opinion (Art. 86(1)(4); Code Art. 28(2)) and to complain.

Ethics and practice

- A reasoned refusal of a **non-indicated** procedure is not paternalism; hiding options or deciding for him would be.
- Explore his goals (pain, work, fear), offer an intensive rehabilitation plan, pain management, review dates.
- Offer a second opinion as support, not as a way of getting rid of him; help him assess the evidence, indication, provider and follow-up of the advertised operation.
- Record the discussion and the reasons.

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Collegiality in the Code of Professional Ethics

> *Collegiality is a basic duty, which must be observed while respecting the interests of the patient.*
>
> — Art. 40

> *It is forbidden to slander a colleague, to speak ill of him and to spread untrue and unproven accusations against him. Personal disagreements between physicians cannot be the subject of public polemics.*
>
> — Art. 41(2)–(3)

> *It is impermissible for a physician to discredit a colleague by assessing and criticising his treatment or by statements against his person.*
>
> — Art. 42

Art.	Content
41(1)	physicians always owe each other moral and professional help
46	ethical disputes between physicians are for the ethics committees of the Bulgarian Medical Association
49	a physician helps a colleague who falls ill and cooperates with those treating him

Collegiality forbids disparagement — not honest information to the patient about findings, alternatives and safety (Health Act, Arts 88, 92).

Consultation, case conference (consilium) and continuity

> *A medical consilium is a meeting of consultant physicians convened by the treating physician at his discretion, at the wish of the patient or his relatives, in cases of difficulty or contradiction in diagnosis and treatment. The decisions of the consilium taken by common consent are binding and are carried out.*
>
> — Code, Art. 38(2)

> *The consultant receives from the treating physician all necessary information about the patient and all available documentation. He documents his conclusion and communicates it to the treating physician and to the patient and/or his relatives.*
>
> — Code, Art. 36(2)

> *... If he withdraws, the treating physician must ensure continuity of treatment by presenting all necessary information to the physician who will replace him.*
>
> — Code, Art. 24(1) (abridged)

> *If, in the absence of the treating physician, the patient has turned to another physician, the latter may attend to the patient and, if necessary, change the treatment ... At the first opportunity he must inform him and enter the change in the medical documentation.*
>
> — Code, Art. 50

Relatives receive information only with the patient's authorisation or another legal basis (Health Act, Arts 28b, 92(4)). A binding consilium decision binds the **team**; carrying it out still needs the patient's informed consent. Gaps in handover are a common cause of preventable harm.

The team and raising concerns

Physician, nurse and team

- each profession has its own competence, professional organisation and code of ethics
- orders are written, legible, checkable; a doubt is **clarified**, not acted on silently
- any team member should call a safety pause and escalate an unresolved concern
- humiliating colleagues before patients destroys trust and hides errors

Situation	Professional conduct
a colleague asks for an informal check-up	a formal consultation, documented, confidential
a colleague's illness affects his work	individual assessment, proportionate limits
a colleague appears impaired before a procedure	stop the procedure, inform the responsible senior, state observations — not diagnoses
misplaced solidarity	silence becomes complicity

Case 4 · The surgeon with a tremor

The case

An experienced surgeon notices a fine hand tremor and asks a neurologist friend to examine him "informally" after work. The examination raises suspicion of a progressive neurological disease. The surgeon refuses documentation and further tests. Tomorrow he has a complex planned operation.

1. What is the status of the "informal" examination?
2. How do confidentiality, collegiality and patient safety fit together?
3. What should the neurologist do — today?

Case 4 · The surgeon with a tremor — key points

Law and Code

- An "informal" examination still creates professional duties: it is documented (Code, Art. 24(1)) and confidential (Health Act, Art. 28c; Code Arts 51–52) — the surgeon has a patient's rights.
- Collegiality is subordinate to the patients' interests (Code, Art. 40).
- A real risk to tomorrow's patient can justify a limited disclosure to the person responsible for the operating list (Health Act, Art. 28(1)(2)), after telling him (Art. 28(2)).

Ethics and practice

- First: a frank conversation; urge him to step back from tomorrow's list and to accept tests.
- If he refuses and the risk is real: disclose **only** the functional concern to the responsible head — not the suspected diagnosis.
- Offer support: confidential care, temporary change of duties, not public exposure.

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Four kinds of legal liability

1 · Disciplinary

Labour Code: disciplinary penalties for culpable breach of work duties; the employee's **financial liability** to the employer (separate basis, Art. 203(3)).

2 · Administrative

administrative violations and fines; subtype: **professional-body liability** before the Bulgarian Medical Association (BMA) / Bulgarian Dental Association (BDA).

3 · Criminal

Penal Code: personal liability for a crime; the **court** decides guilt and punishment.

4 · Civil

Obligations and Contracts Act: **compensation**; contractual and tort (non-contractual).

One event may engage several kinds, **subject to each regime's conditions and procedural limits**: e.g. once criminal prosecution is opened, no administrative-penal proceedings are opened for the same act, and signs of a crime end them with referral to the prosecutor (Administrative Violations and Penalties Act, Art. 33).

	Aim	Who decides	Fault
Disciplinary	labour discipline	employer	culpable breach
Administrative	lawful functioning of public administration	competent authority	culpable violation (persons); pecuniary sanction (entities)
Criminal	punishment, prevention	court	intent or negligence (Art. 11)
Civil	repair the harm	court, on a claim	presumed in tort (Art. 45); harm and causation must be proved

1 · Disciplinary liability (Labour Code)

> *Culpable failure to perform work duties is a breach of labour discipline. The offender is punished with the disciplinary penalties provided for in this Code regardless of financial, administrative-penal or criminal liability, if such liability is provided for.*
>
> — Labour Code, Art. 186

Penalties (Art. 188): reprimand · warning of dismissal · dismissal. **Procedure:** imposed by the employer (Art. 192) after hearing the employee or receiving written explanations and assessing the evidence (Art. 193(1)); otherwise the court annuls the penalty (Art. 193(2)) — unless the explanations were missing through the employee's fault (Art. 193(3)).

> *The employee is financially liable ... for the harm he has caused to the employer through negligence in or in connection with the performance of his work duties. ... Financial liability applies regardless of disciplinary, administrative-penal and criminal liability for the same act.*
>
> — Labour Code, Art. 203(1) and (3)

Negligence: up to one monthly salary (Art. 206(1)); for managerial functions up to three (206(2)); the same limits when the employer has compensated a third party (206(3)). Intentional harm or harm from a crime: civil law (203(2)). Normal operational risk: no liability (204).

2 · Administrative and professional-body liability

> *An administrative violation is an act (action or omission) that violates the established order of state administration, is committed culpably and is declared punishable with an administrative penalty imposed by administrative procedure.*
>
> — Administrative Violations and Penalties Act, Art. 6

Health Act: Art. 221 — an establishment that violates patients' rights is fined (para. 1) or, as a sole trader or legal person, receives a **pecuniary sanction** (para. 2); the violation is established by officials of the **Medical Supervision Executive Agency** and its executive director issues the penalty decree (Art. 233a). Art. 229 — residual penalty for other breaches of the Act.

> *Physicians and dentists are liable for the following violations committed in the performance of their professional duties: 1. non-compliance with the rules of the codes of professional ethics; 2. non-compliance with the rules of good medical practice.*
>
> — Professional Organisations Act, Art. 37(1)

Penalties for code breaches (Art. 38): reprimand · fine of one to five minimum monthly wages · removal from the register for 3 months to 1 year. Ethics committee hears complaints (Art. 39); its member draws up the act; the **chair of the regional board** issues the decree (Art. 40); appeal under the Administrative Violations and Penalties Act (Art. 41). Other liability is not excluded (Art. 42(1)).

3 · Criminal liability (Penal Code)

> *A person who causes the death of another through ignorance or negligent performance of an occupation or other legally regulated activity that is a source of increased danger shall be punished by imprisonment of one to six years.*
>
> — Penal Code, Art. 123(1)

Art. 134(1) covers **grievous or moderate bodily injury** caused by the same professional misconduct; its penalties are lower. Art. 11: guilt is intent or negligence; negligent acts are punishable only where the law expressly provides (Art. 11(4)).

> *A person practising a medical occupation who, having been called, does not come to the aid of a patient or a woman in labour without a valid reason, shall be punished ... If the offender knew that the patient was in a dangerous condition, the punishment is imprisonment of up to one year or probation.*
>
> — Penal Code, Art. 141(1)–(2) (abridged)

Arts 123 / 134: what must be proved

the specific breach of a professional rule · death or injury · the **causal link** · fault. An adverse outcome alone is not a crime. Art. 141 has different elements: being called, failing to help, no valid reason.

4 · Civil liability (Obligations and Contracts Act)

Contractual (Arts 79, 82)

failure to perform a contractual obligation properly; damages cover the loss suffered and the profit lost that are a **direct and immediate** consequence and could be **foreseen when the obligation arose** — all direct and immediate losses if the debtor acted in bad faith (Art. 82).

Example: an agreed and paid examination is not performed and the patient incurs proven additional costs.

> *Compensation for non-pecuniary harm is determined by the court on an equitable basis.*
>
> — Art. 52

> *Everyone is obliged to repair the harm he has culpably caused to another. In all cases of tort, fault is presumed until proven otherwise.*
>
> — Art. 45

> *A person who has assigned work to another is liable for the harm caused by him in or in connection with the performance of that work.*
>
> — Art. 49

Example: an incorrectly given injection damages a nerve; the patient sues the physician and the hospital. Compensation **may** be reduced where the injured person's contribution to the harm is established (Art. 51(2)).

The European Court of Human Rights and medical negligence

Judgment	Principle	Outcome
Calvelli and Ciglio v Italy [GC], no. 32967/96, 17.01.2002	for unintentional harm in health care, an effective **civil** remedy may suffice; criminal proceedings are not required in every case (disciplinary measures may also be envisaged)	no violation
Šilih v Slovenia [GC], no. 71463/01, 09.04.2009	after a death in hospital the State must provide an **effective and prompt** judicial system to establish the cause and responsibility	procedural violation of Art. 2
Lopes de Sousa Fernandes v Portugal [GC], no. 56080/13, 19.12.2017	where an effective regulatory framework exists, ordinary negligence does not in itself breach Art. 2; State responsibility arises only in exceptional cases (knowingly denying life-saving emergency care; systemic dysfunction) under strict conditions — but proceedings must be effective	no substantive violation; procedural violation of Art. 2
Vasileva v Bulgaria, no. 23796/10, 17.03.2016	a Plovdiv patient sued after rib fragments were removed from sites different from those on imaging; the Court reviewed the **domestic process**, not the operation: proceedings need **independent, reasoned** expert evidence that the courts scrutinise	no violation of Arts 8 and 6

The Convention does not guarantee a favourable outcome — it guarantees effective proceedings and independent experts.

Case 5 · A handover without the new symptom

The case

On 24 December, at the end of his shift, a physician waits almost an hour for a late colleague. A nurse reports chest pain in a patient admitted two days earlier with pneumonia. The physician orders an ECG and says the next doctor will assess him. In the rushed handover he forgets to mention the symptom. That night the patient is transferred to coronary care with a large myocardial infarction and later dies.

1. Who owed a duty of care to the patient at the moment of the call?
2. Apply the four kinds of liability: which may arise, and for whom?
3. What share belongs to the organisation?

Case 5 · A handover without the new symptom — key points

Liability, kind by kind

1. **Disciplinary:** a handover is a work duty (Labour Code, Arts 186–187); the late colleague separately.
2. **Administrative (professional body):** breach of continuity of care (Code, Art. 24(1)) and good medical practice (Professional Organisations Act, Art. 37).
3. **Criminal:** Penal Code Art. 123 only if breach, causation and fault are proved — an adverse outcome is not enough.
4. **Civil:** tort (OCA Art. 45 — fault presumed, but harm and causation must be proved); the hospital answers for its staff's culpable conduct in the assigned work (Art. 49).

Ethics and system

- He was responsible for the patient throughout the shift: new chest pain calls for prompt assessment and ECG review; responsibility passes only through an effective handover to an identified clinician who accepts it.
- System factors to **investigate**: handover structure, cover for late arrivals, workload; assess the late colleague and each causal contribution separately — the death does not by itself prove causation.
- The lesson is a safer process (structured handover tools, written handover of "watch" patients), not only a culprit.

Case 6 · The doctor in the supermarket

The case

A young doctor, three months into his job, goes shopping after a 12-hour night shift. Someone shouts for help: a person has collapsed and has a bleeding head wound. He introduces himself as a doctor, checks breathing, presses on the wound and asks for an ambulance; the paramedics take over. A colleague later warns him: "You could be sued."

1. Was there a legal duty to help? Is it different for a doctor?
2. What standard of care applies in a supermarket?
3. Should fear of liability make a doctor walk past?

Case 6 · The doctor in the supermarket — key points

Law

- Everyone must help a person in immediate danger to life when he can do so without danger to himself or others (Penal Code, Art. 139).
- A person practising a medical occupation who is **called** and does not help a patient without a valid reason commits a special offence (Art. 141); Code Art. 11 — emergency help regardless of specialty.
- Civil liability would require culpable harm (OCA Art. 45); reasonable first aid with the means available is not negligence.

Ethics

- Beneficence and the profession's public trust support helping; the standard is that of a reasonable doctor **in those conditions**, not of an equipped hospital.
- Act within competence, call for help, hand over clearly to the paramedics, write a short note.
- Helping creates no automatic immunity, and inability to help safely is assessed in context — but reasonable help within competence is what the law and the profession expect.

Case 7 · The expensive watch

The case

Dr N. has cared for Mr V., a wealthy patient with chronic lung disease, for twelve years. At the end of a visit he offers her a very expensive watch: refusing would insult him. She accepts it privately. A week later he asks her for a certificate stating that delay of a commercial permit would put his life at immediate risk — the clinical findings do not support that.

1. What makes this gift different from a card or flowers for the ward?
2. What should she do about the certificate — and the watch?
3. Which kinds of liability could a false certificate engage?

Case 7 · The expensive watch — key points

Ethics

- High value, secrecy and a later request turn gratitude into **reciprocity**: the gift now threatens her judgement and public trust.
- Refuse the certificate; explain politely; return the watch or hand it over through the institution's procedure; record what happened.
- Keep caring for him — the relationship is preserved by honesty, not by the gift.

Law

- A physician who **knowingly** provides someone with a false certificate about his state of health, when not acting as an official, commits an offence (Penal Code, Art. 312(1)); an official who certifies untrue facts in an official document for use as evidence — Art. 311 (official status under Art. 93(1)). A mistaken statement is not a crime. Professional liability: Professional Organisations Act, Art. 37(1).
- The employer's internal rules on gifts and conflicts of interest apply (disciplinary liability, Labour Code, Art. 187(1)(10)).

Summary

Relationship

1. Chronic care needs **agreed** goals, attention to treatment burden and relational autonomy.
2. Models: Szasz–Hollender (participation), Veatch (ethical authority), Emanuel & Emanuel (patient's values); Emanuel and Emanuel favour the deliberative model as the general ideal.
3. Honesty, shared decisions and clear boundaries protect trust; collegiality never outranks the patient (Code, Art. 40).

Liability

1. Four kinds: disciplinary (Labour Code) · administrative incl. professional body · criminal (Penal Code) · civil (OCA).
2. They are independent; one event may engage several, each under its own conditions and procedural limits.
3. The ECtHR requires effective proceedings and independent experts — not a guaranteed result.

Trust is built in the relationship, the team and the institution; the law sets out the consequences when it is broken.

Thank you for your attention!